

Patient:

Referring Physician:

UUID:84221F34-EFA9-4ADD-A483-6B419D8A025B
TCGA-GC-A6I3-01A-PR

Redacted



DOB:

Age

Gender: F

Ref#:

Hosp#:

Provider Group:

Date of Service:

Date Received:

Inpatient

Case #:

Date Reported

FINAL SURGICAL PATHOLOGY REPORT

Diagnosis:

ICD-O-3
Concurrence, urothelial NOS
8/20/13
Site Bladder NOS
C67.9
9/6/12/13

A. - H.) BLADDER, UTERUS, BILATERAL FALLOPIAN TUBES AND OVARIES, LEFT AND RIGHT OBTURATOR LYMPH NODES, SEGMENTS OF LEFT AND RIGHT URETER AND DISTAL URETER, AND APPENDIX; RADICAL CYSTECTOMY WITH URETHRECTOMY, BILATERAL PELVIC LYMPH NODE DISSECTION, HYSTERECTOMY WITH BILATERAL SALPINGO-OOPHORECTOMY, BILATERAL URETEROTOMY, AND APPENDECTOMY:

- INVASIVE HIGH-GRADE UROTHELIAL CARCINOMA WITH FOCAL SQUAMOUS DIFFERENTIATION, 7.3 CM IN GREATEST DIMENSION.
- TUMOR INVADDES THROUGH MUSCULARIS PROPRIA INTO PERIVESICAL ADIPOSE TISSUE.
- TUMOR INVOLVES MAINLY THE ANTERIOR AND LEFT LATERAL BLADDER WALL; TUMOR ALSO FOCALLY EXTENDS TO POSTERIOR WALL (ADJACENT VAGINAL WALL IS NEGATIVE FOR TUMOR).
- FOCAL LYMPHOVASCULAR INVASION IDENTIFIED.
- METASTATIC CARCINOMA IDENTIFIED IN ONE OF SEVENTEEN REGIONAL LYMPH NODES (1/17).
 - Metastasis involves a left obturator lymph node and measures 0.7 cm in greatest dimension; no extranodal extension identified.
- ALL MARGINS (DISTAL URETHRAL, DISTAL VAGINAL, RADIAL, AND URETERAL MARGINS) ARE NEGATIVE FOR TUMOR.
- BENIGN SEPARATELY SUBMITTED SEGMENTS OF BILATERAL URETERS (PARTS A, B, G, AND H).
- BENIGN CERVIX AND UTERUS WITH INACTIVE ENDOMETRIUM WITH CYSTIC ATROPHY; UNREMARKABLE MYOMETRIUM.
- BENIGN BILATERAL OVARIES WITH SURFACE INCLUSION CYSTS AND PHYSIOLOGIC CHANGES.
- BENIGN BILATERAL FALLOPIAN TUBES WITH PARATUBAL CYSTS.
- BENIGN APPENDIX.

per TSS, no squamous
component identified by
TSS pathologist.
BCK

PATHOLOGIC TUMOR STAGING SYNOPSIS:

Type and grade (invasive): Invasive high-grade urothelial carcinoma with focal squamous differentiation.

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Type and grade (In situ): Not identified.

Primary tumor: pT3a.

Regional lymph nodes: pN1.

Distant metastasis: N/A.

Pathologic stage: IV.

Lymphovascular invasion: Present.

Margin status: R0, negative.

(AJCC Cancer Staging Handbook, 7th Ed, and CAP protocol, June 2012)

Previous biopsy:

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Specimen:

Procedure:

Tumor size:

Histologic type:

Associated epithelial lesions:

Histologic grade:

Microscopic tumor extension:

Margins:

Lymph-vascular invasion:

Lymph nodes:

Pathologic Staging:

Primary tumor

Regional lymph nodes

Distant metastasis

Pathologic stage

Bladder, uterus, bilateral fallopian tubes and ovaries, right and left obturator lymph nodes, segments of left and right ureters, and appendix.

Radical cystectomy with urethrectomy, bilateral pelvic lymph node dissection, hysterectomy with bilateral salpingo-oophorectomy, bilateral ureterotomy, and appendectomy.

7.3 x 6 x 1.8 cm.

Urothelial carcinoma.

N/A.

High-grade.

Tumor invades through muscularis propria into perivesical adipose tissue.

All margins (distal urethral margin, distal vaginal margin, radial margin, and bilateral ureteral margins) are negative for tumor.

Focally present.

Metastatic carcinoma identified in one of seventeen regional lymph nodes (1/17).

pT3a.

pN1.

N/A.

IV.

Source of Specimen:

A. Right ureter

B. Left ureter

C. Ovary and fallopian tube, Uterus; Bilateral tubes and bladder

D. Lymph node; Right obturator lymph node

E. Lymph node; Left obturator lymph node

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F. Appendix
G. Left distal ureter
H. Right distal ureter

Clinical History/Operative Dx:

Bladder cancer

Intraoperative Diagnosis:

A. Right ureter: No tumor or dysplasia seen.
B. Left ureter: No tumor or dysplasia seen
performed and rendered at

Gross Description:

A. Part A is right ureter. Initially received in the fresh state for frozen section analysis is a single light tan tubal fragment of soft tissue, 4 mm in length and 4 mm in diameter. The surgical margin is marked blue. The specimen is entirely submitted for frozen section analysis with the residual frozen tissue submitted in A1 for permanent sections.

B. Part B is left ureter. Initially received in the fresh state for frozen section analysis is a light tan tubal fragment of soft tissue, 5 mm in length and 4 mm in diameter. The narrow 1.5 mm lumen is patent. The surgical margin is marked orange. The specimen is entirely submitted for frozen section analysis with the residual frozen tissue submitted in B1 for permanent sections

C. Part C is designated as uterus, bilateral tubes and ovaries, bladder. Initially received in the fresh state for possible is a generous portion of tissue consisting of a bladder-radical cystectomy and a hysterectomy specimen with extensions of urethra and vaginal tissue. The specimen weighs in aggregate 343 grams and the bladder is 8.0 x 7.2 x 4.2 cm with a 4.0 cm extension of urethral tract, 1.4 cm in diameter. The attached vaginal vault has been previously incised lengthwise measuring 9.2 cm in length and up to 3.0 cm in diameter. The uterus is 5.8 cm in length, 4.0 cm right to left lateral and 2.3 cm posterior-anterior. The bilateral fallopian tube segments and ovaries are intact. The peritoneal reflection is glistening, smooth, light tan and transitions to deep red over a palpable mass within the bladder.

The surgical margins-surfaces are now differentially inked as follows: posterior margin of the length of the introitus is marked black as well as the uterine serosa, the distal margin of the vagina is marked orange, the anterior margin of the perivesicular fat is marked blue and the distal margin of the urethral tract is marked orange.

Initially, the bladder is incised in a Y-shaped fashion and opened to reveal a large, dense to shaggy and delicate, red-tan tumor mass measuring upwards of 7.3 x 6.0 cm and grossly arising mid anterior through the left lateral and dome of the fundus. The tumor advances inferiorly along the lateral mucosa to the bladder neck, at the edge of the urethral orifice, 4.0 cm from the distal urethral surgical margin. Initial sectioning through the tumor mass demonstrates gross full thickness involvement of the bladder wall, anteriorly. Representative portions of tumor, para-neoplastic and normal bladder mucosa are submitted for

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The tumor mass grossly approaches within 0.9 cm of the left ureteral orifice which is probe patent to a 2.4 cm length of ureter. The ureter is probe patent and is up to 3.0 cm in length and 0.4 cm in diameter. The remainder of the bladder mucosa demonstrates a glistening, smooth to slightly bosselated-wrinkled, light tan and pink erythematous appearance without additional discrete lesion. The length of the urethra is glistening, erythematous, pink and tan without grossly discrete lesion.

Examination of the vaginal vault reveals glistening, smooth to wrinkled, edematous, pink and tan mucosa with a 0.7 cm focus of sloughing, mid-length. Initially, there is no obvious tumor involvement. The ectocervix is up to 3.3 cm in diameter with petechial congestion and a 1.0 cm os. The squamocolumnar junction is poorly demarcated with the endocervical component predominantly glistening, smooth to slightly trabecular. The bisected uterus demonstrates a triangular endometrium, 3.0 x 1.6 cm, and flattened to less than 0.1 cm with several minute, clear fluid-filled cysts. The myometrium is rubbery, pink and tan, without discrete nodularity or unusual areas of change suggestive of neoplasia.

The right fallopian tube with fimbria is 5.5 x 0.4 cm, hyperemic, pink and tan, with a delicate lumen. The 1.8 x 1.2 x 0.7 cm ovary is cerebiform, yellow-tan. The bisected ovary demonstrates atrophic, rubbery to indurated parenchyma with a few small cysts, up to 0.4 cm, with straw colored mucin. No discrete nodularity appreciated. The left fallopian tube with fimbria is 5.6 x 0.4 cm, hyperemic, pink and tan, with a delicate lumen. The 1.4 x 1.1 x 0.8 cm ovary is cerebiform, yellow-tan. The ovary is trisected, to reveal soft to indurated, atrophic, yellow tan parenchyma. There are no discrete nodularities or unusual areas of change, suggestive of neoplasia.

Examination of the perivesicular fat reveals four (4) discrete lymph node candidates, ranging from 0.2 to 0.6 cm each.

Cassette summary:

- C1) distal vaginal margin, posterior,
- C2) distal urethral margin and anterior distal vaginal margin,
- C3) right and left (orange) ureter surgical margin,
- C4-C6) tumor-nearest approach to vaginal wall, left lateral, at bladder base,
- C7) tumor, left ureter and vaginal wall relationship,
- C8) left ureteral orifice and vagina, full thickness,
- C9-C11) tumor-left lateral perivesicular fat, including radial surgical margin,
- C12-C13) tumor-anterior lateral radial perivesicular margin, four pieces,
- C14-C15) tumor, mid anterior-lateral fundus and peritoneal reflection,
- C16) dome of fundus, two pieces,
- C17) mid posterior bladder,
- C18) right lateral bladder,
- C19) right ureteral orifice, and length of ureter, two pieces contiguous sections,
- C20) proximal and distal urethra-vaginal length, contiguous sections,
- C21) posterior cervix and vagina, contiguous sections, additional piece of distal end of vagina,
- C22) anterior cervix and vagina, contiguous sections,
- C23-C24) posterior endomyometrium, three pieces,
- C25-C26) anterior endomyometrium, three pieces,
- C27) right fallopian tube, including fimbria, represented,

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C28) right ovary bisected and submitted,
C29) left fallopian tube, including fimbria, represented,
C30) left ovary trisected and submitted.
C31) one perivesicular lymph node candidate,
C32) two perivesicular lymph node candidates,
C33) one perivesicular lymph node candidates

D. Part D designated right obturator node. Received in formalin are multiple lobular and tattered pieces of fatty soft tissue measuring in aggregate of 5.4 x 4.3 x 1.7 cm. Examination reveals ten lymph node candidates ranging from 0.5 x 0.4 x 0.3 cm to 2.3 x 1.7 x 1.4 cm each. The lymph nodes are rubbery, light pink and tan. Sectioning through two of the lymph nodes demonstrates a suspicious dense pink-gray appearance, D2, D5-D6. The lymph nodes are entirely submitted for microscopic evaluation.

Cassette summary: D1) five lymph nodes, D2) two lymph nodes bisected, one marked orange, D3) single lymph node, bisected, D4) single lymph node three pieces, D5-D6) largest lymph node trisected.

E. Part E designated left obturator lymph node. Received in formalin is a lobular and tattered portion of yellow-tan fatty soft tissue, 9.3 x 8.8 x 1.6 cm. Examination reveals up to six lymph nodes, three smallest of which range from 0.6 x 0.5 cm to 0.8 x 0.7 cm. The three largest are elongated rubbery suspiciously rubbery to firm pink-gray, ranging from 3.0 x 1.7 x 1.2 cm, to 4.3 x 1.9 x 1.4 cm each. The lymph nodes are entirely submitted for microscopic evaluation.

Cassette summary: E1) three lymph nodes, E2-E3) single lymph node candidate, trimmed and submitted, E4-E5) single lymph node sectioned and submitted, E6-E9) largest lymph node sectioned and submitted.

F. Part F designated appendix. Received in formalin is a vermiform appendix, 7.3 x 0.5 cm. The serosa is hyperemic pink and tan without discrete nodularities. The lumen is slit-like 2-3 mm in diameter. The apex demonstrates glistening light pink and tan thickening over an area of 0.5 x 0.4 cm. There is no fecalith. Representative sections are submitted to include representation of proximal surgical margin (blue), mid length, and bisected apex submitted in F1.

G. Part G designated left distal ureter. Received in formalin is a pink and tan tubular fragment of soft tissue, 5 mm in length and 6 mm in diameter. One end is crimped by a metallic clip (blue). The lumen is patent 3 mm in diameter. The specimen is entirely submitted, bisected, in G1. A tube tab is inserted within the cassette.

H. Part H designated right distal ureter. Received in formalin is a light pink and tan tubular fragment, 6 mm in length, and 6 mm in diameter. One end of the specimen is crimped by metallic clip (blue). The specimen is bisected and entirely submitted in H1. A tube tab is inserted within the cassette.

Microscopic Description:

A. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

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B. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

C. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered. Sections show invasive high-grade urothelial carcinoma with focal areas of squamous differentiation. The tumor invades through muscularis propria into perivesical adipose tissue. The tumor involves predominantly the anterior wall and extends towards the left lateral wall. The tumor focally involves the posterior wall without involvement of the adjacent vaginal wall. See diagnosis and synoptic report for summary.

Intradepartmental consultation:
slides and concur.

have reviewed selected

D. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

E. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

F. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

G. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

H. Microscopic sections have been examined. The microscopic findings are reflected in the diagnosis rendered.

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HUPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Malignancy		
Case is (clinical) QUALIFIED / DISQUALIFIED		
Reviewer Initials	Date Review: 6/10/13	

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